

إسم المريض رابعياً : عليه عبدالسلام سيد نصر

العمر(تاريخ الميلاد) : 03-06-1958 المهنة : بدون عمل

المقر بما فيه
التوقيع: (-----)

التعليق على التشخيص: -----

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

التشخيص النهائي :
 خطة العلاج :
 المستشفى :
 قسم :
 جهة العلاج :
 نفقات :
 ملاحظات :

رئيس
المجلس الطبي المتخصص

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Name: عليه عبدالسلام سيد نصي National ID: 25806032100065	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد انه
--

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Zoledronic acid for bone protection during hormonal therapy
--

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

جمهورية مصر العربية
بطاقة تحقيق الشخصية

عليه
عبد السلام سيد نصر
عبد السلام المنسي كفر طهر من الجبل
بولاق الدكرور - الجيزة

١٩٥٨/٠٦

٢٥٨٠٦٠٣٢١٠٠٠٦٥

LM6795055

٢٠٢٥/١١ ٢٥٨٠٦٠٣٢١٠٠٠٦٥
بدون عمل

أنثى مسلمة أرملة

البطاقة سارية حتى ٢٠٢٢/١١/٢٨

١

Patient Name	عليه عبد السلام سيد نصر 0	Code Number	YK 262930
Sex/Age	Female 67Y	Patient ID	20105
Registered Date	23/05/2026 14:24:38	Referred By Dr.	Surgery
Validation Date	20/06/2026 09:10:04	Branch	Gustave Roussy
Reporting Date	6/20/2026, 10:27:04 AM		

Pathology Report

Pathology Report

Clinical Data: Left breast cancer, post-NACT.

Nature of specimen: Left modified radical mastectomy.

Gross Examination:

- * Left modified radical mastectomy specimen measured 28x18x5 cm, with covering skin ellipse measured 24x12 cm. Nipple & areola looked unremarkable grossly.
- * Serialling showed a tumor bed site 5x2.5x2 cm, located 1 cm from the least deep margin.
- * Axillary fat measured 8x7x2 cm, entangling 10 lymph nodes, the largest 1.5x1x0.5 cm.
- * Block designation:
- * 11 Blocks:
- 1:5- T.B.S., 6- N&A., 7- Deep margin., 8:11- ALNs.
- Gross was done by; Dr. Heba Hassan.

Microscopic examination:

Histological type: No residual viable remnants of the previously diagnosed invasive tumor. (Cellularity 0%).
Ductal carcinoma in situ: Absent.
Angioinvasion: Absent
Micro-calcification: Present.
Nipple and areola: Free.
Deep margin: Free.
Axillary lymph nodes: Ten lymph nodes free of metastatic deposits (0/10) (without therapy effect)
Histologic response to therapy: Pathological complete response to therapy.

Diagnosis:

Left Breast Cancer (Post-NACT); Left Modified Radical Mastectomy:

- No residual viable remnants of the previously diagnosed invasive tumor. (Cellularity 0%).
 - Ductal carcinoma in situ: Absent.
 - Nipple and areola: Free.
 - Deep margin: Free.
 - Axillary lymph nodes: Ten lymph nodes free of metastatic deposits (0/10) (without therapy effect).
 - Histologic response to therapy: Pathological complete response to therapy.
- Stage: ypT0, ypN0.

Signature

Dr. Walaa Ibrahim Ghoneim

العنوان : 987 كورنيش النيل مصر القديمة - رقم خدمة : 15580 / 0221277647

Patient Name

Sex/Age

Registered Date

Validation Date

Reporting Date

عليه عبد السلام سيد نصر 0
Female 67Y
23/05/2026 14:24:38
20/06/2026 09:10:04
6/20/2026, 10:27:04 AM

Code Number

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Referred By Dr.

Branch

YK 262930
20105
Surgery
Gustave Roussy



غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

العنوان : 987 كورنيش النيل مصر القديمة - رقم خدمة : 0221277647 / 15580



مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

الاستشارة
معدة

04/29/2026

MDT FORM

Name:	Alyia Abd Elsalam Sayed .	MRN:	687
Age:	68 Y	ID:	25806032100065
Menopausal Sta	POSTM	Phone:	01021508718
NRS SCORE:	PS2	Marital Status:	
BMI:		Family History:	
CO Morbidity:	HTN	Reason Of MDT:	

Clinical Data:

BX:	IDC						
ER:	6/8	PR:	3/8	HER2:	3+	KI67:	

R

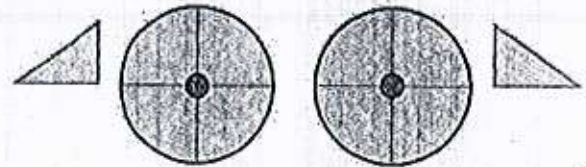
L

Sonomammography:Mets Workup:History:

67 Y WT 80 HT160 SA 1.8 POSTM LEFT BREAST CANCER CT3 N2 MO ER6/8 PR3/8 HER23+ PT WAS DIAGNOSED IN 2024 BUT DELAYED BEC OF CAR ACCIDENT (RT FEMORAL FRACTURE AND INT FIXATION) CTS 11/2025 FREE BS 11/2025 ACTIVE UPTAKE AT STERNUM AND SC J FOR CT CORR CTS FREE RT FEMORAL TROCHANTER LESION (CAR ACCIDENT) PT RECEIVED 6 TAXOTERE CARBOPLATIN LAST 18/4/2026 3 PHESGO WITH REGRESSIVE COURSE FOR MDT FOR SURGERY

Recommendations:MDT Outcome & Plan

for simple mastectomy and axilla according to response



TNM

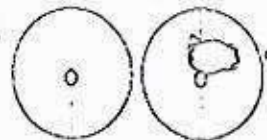
SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
د/خديجة حسن	د/عبد السلام شامي	د/ترمين رمضان			



MDT form

A-Patient ID: 1- Name : عليہ عبدالسلام سيد نصر Sex: Female 3- Medical number: 83253140 Old Med: 43-2025
2-ID number: Patient Age : 67 Y



Family history	Family History -ve	Phone	Phone 01122811410	BMI	BMI	REASON OF MDT	REASON OF MDT N/A
marital status	Marital Status Divorced	HRS Score	HRS Score	GROUP	GROUP	Date of MDT	Date of MDT 03/02/2025
Co morbidity	Co Morbidity hta	Menopausal status	Menopausal Status Postmenopausal	Initial STAGE	Initial STAGE		

Clinical data

Breast Imaging : It breast UOQ area of pleomorphic calcification with underlying focal syr 9.5x5 cm multiple pathological LN	Mets Workup : bulk uterus with thickened endometrium
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BX IDC BX COMMENT:	ER 6/8 ER COMMENT:	PR 3/8 PR COMMENT:	Her 2 Neu 3 Her 2 Neu COMMENT: +ve	SISH SISH COMMENT:	Ki67 35
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T CT3 M	N 1	M 0
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Sonomammography

PATIENT HISTORY Pt has been diagnosed with breast cancer (DCIS) and is being treated with NACT. She has a history of hypertension and is on medication. She is currently on NACT and has a good response. She is being followed up with ultrasound and mammography. She is also being followed up with a gynecologist. She is currently on NACT and has a good response. She is being followed up with ultrasound and mammography. She is also being followed up with a gynecologist.

LABS, ECHO Free only uptake on breast at side of tumor. To start NACT TCHP X6

D&C

BONE SCAN

MDT OUTCOME & PLAN

MDT OUTCOME & PLAN
NACT TCHP X6 THEN LT MASTECTOMY

SIGNATURE

MEDICAL ONCOLOGY [Signature]	RADIOLOGY [Signature]
SURGERY [Signature]	PATHOLOGY [Signature]
Psychiatric [Signature]	Nutrition [Signature]

Patient Name : الأ. / ش. / م. / د. / ...
Patient No. : 25806032100065
Path No. : 4747/24
Sex : Female
Age : 66 Y
Referring Doc :

Request No. : 13002353
Request Date : 03/12/2024
Verification Date : 09/12/2024
Financial Category : ...
Department : Out Patient
Registered by : Mrs. Mayada Hegazy

Pathology Report

Clinical data:
Left breast mass.

Nature of specimen:
Tru cut biopsy, collected at 2/12/2024.

Gross examination
Three greyish white tissue cores up to 1.5 cm , totally submitted.

Microscopic examination:

- * Histological type: Invasive duct carcinoma .
- * Grade (differentiation +nuclear pleomorphism+mitosis): (3+2+1) Grade II .
- * DCIS:Focal.
- * Angioinvasion: Could not be detected within the limits of biopsy recieved .

Diagnosis:

Left breast mass, tru cut biopsy:
Invasive duct carcinoma, Grade II .
For further IHC (ER,PR,HER2,Ki67).

Prof. Gamal Abdelatty
Dr. Noura Hefny

Verified by :
dr.esraa magdy

Printing Date : 21/12/2024 09:35:25

Page 1 of 1

Printing By : marwa mohamed youss

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Room No. (4) - First Floor - Building No. 987 - Nile Corniche - Nasser El Qadima - Cairo, Egypt

Patient Name : الأستاذ / عتيه عبد السلام سيد نصر
Patient No. : 25806032100065
Path No. : 4747/24
Sex : Female
Age : 66 Y
Referring Doc. :

Request No. : 13002353
Request Date : 03/12/2024
Verification Date : 25/12/2024
Financial Category : اورام دار السلام - مبادره
Department : Out Patient
IHC No. :
Registered by : Mrs.Mayada Hegazy

IMMUNOHISTOCHEMISTRY REPORT

Clinical data: Left breast mass.
Nature of specimen:
Tru cut biopsy, collected at 2/12/2024.

Technique:

- Sections were prepared from the paraffin block, processed in Bench Mark ULTRA machine (Ventana) and stained by rabbit monoclonal antibodies using ultra view HRP DAB as chromogen and hematoxylin as counter stain. The sections treated against:
- ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Results:

- IPT (Positive control showed appropriate reactivity):
- ER: Strong nuclear reaction in 15% of tumor cells (score 6/8).
- PR: Weak nuclear reaction in 5% of tumor cells (score 3/8).
- Her2: Strong complete membrane reaction in >10% of tumor cells (score 3+).
- Ki67: 35% nuclear reaction of tumor cells.

Conclusion:

Left breast mass, Tru-cut biopsy:
Invasive duct carcinoma, Grade III.

IPT:

- ER: Positive (score 6/8).
- PR: Positive (score 3/8).
- Her2: Positive (score 3+).
- Ki67: Positive in 35% of tumor cells.

Scoring system: "ER, PR"

Verified by :
Dr.Toka Medhat

Printing Date : 05/01/2025 09:30:38

Page 1 of 2

Printed By : Mrs.Mayada Hegazy

Patient Name	الأستاذة / عليه عبد السلام سيد نصر	Request No.	13002353
Patient No.	25806032100065	Request Date	03/12/2024
Path No.	4747/24	Verification Date	25/12/2024
Sex	Female	Financial Category	اورام دار السلام - مبادره
Age	66 Y	Department	Out Patient
Referring Doc.		Registered by	Mrs.Mayada Hegazy

IMMUNOHISTOCHEMISTRY REPORT

ASCO-CAP ER/PR Test Guideline Recommendations, 2010 Allred score: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak, 2, intermediate; and 3, strong). Total score from 0 to 8.

Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013. IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within < 10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is incomplete, intense < 10% of tumor cells.

Signature
Dr. Salma Tarek
Dr. Duaa Mohamed
Dr. Yasmine Essam

Verified by :
Dr.Toka Medhat

Printing Date : 05/01/2025 09:30:38

Page 2 of 2

Printed By : Mrs.Mayada Hegazy

CAP
ACCREDITED

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Room No. (4) - First Floor-Building No. 987- Nile Corniche - Mizr El Qadema - Cairo, Egypt

Name : ALIAA ABDEL SALAM SAYED

Date : 07-02-2026 12:19

Branch : Portable

Medical No. : GRI-94759

Ref.Physician Prof : TRIPHASIC

Ins.Company :

**MULTI-SLICE CT SCAN OF THE ABDOMEN AND PELVIS AFTER
BOTH ORAL & I.V. CONTRAST ADMINISTRATION TRIPHASIC:**

Findings:

In comparison to previous study dated 8/11/2025 current study reveals:

- The uterus is mildly bulky with distended cavity, right adnexal cyst and dilated right fallopian tube with fluid inside ?Salpembococle for US correlation.
- Small fatty umbilical hernia.
- The liver is of average size with homogenous CT texture. No focal lesions or contour irregularities.
- Patent homogeneously enhancing portal vein and its main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.
- Clear scanned lung bases.

Opinion:

- The uterus is mildly bulky with distended cavity, right adnexal cyst and ?Salpembococle for US correlation.

Dr Mai Mohamed MD

Name : Alia Abdul Salam Sayyid Nasr

Date : 29-12-2025 10:01

Branch : Nasrcity

Medical No. : 248576

Ref.Physician Prof : Goustav Roussey Egypt

Ins.Company : التأمين الصحي

..Whole body PET/CT EXAMINATION

Clinical History:

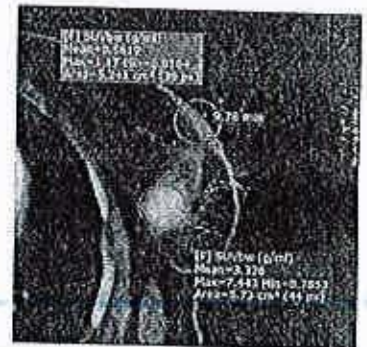
- 67 years old female patient diagnosed with left cancer breast received chemotherapy. PET/CT requested for initial assessment.

Technique:

- For the aforementioned patient we have done a total body PET scan one hour after IV injection of 370 MBq of 18 F-FDG. MSCT was performed using GE Discovery IQ simultaneously and used for attenuation correction, anatomical localization and diagnosis. Max. variant of SUV; a Semi-quantitative analysis would be done for selected ROI.s and the normal threshold is < 2.5.

Report:

- Left breast lower outer FDG avid metabolically active irregular mass is noted averaging 3x2.5 of SUV max 7.4 as well as related lower outer FDG avid parenchyma of SUV max 2.5 . this is associated with low FDG avid thickened skin reaching 1 cm of SUV max 1.**
- Locoregional:** This is associated with locoregional level I left axillary single FDG avid lymph node averaging 2.5 cm of SUV max 9.4 .
- Distant:** Liver shows heterogeneous FDG uptake with segment III foci of SUV max 5 with no corresponding non contrast CT finding...for further Triphasic CT/MRI
- Otherwise no other** No distant FDG avid metastatic deposits could be noted.



Name : Alia Abdul Salam Sayyid Nasr

Date : 29-12-2025 10:01

Branch : Nasrcity

Medical No. : 248576

Ref.Physician Prof : Goustav Roussey Egypt

Ins.Company : التأمين الصحي



Head and Neck:

- Normal diffuse brain activity with no hypermetabolic brain lesions.
- Physiologic FDG uptake is seen in the nasopharynx, oropharynx, salivary glands and larynx, showing normal appearance.
- Physiologic FDG uptake is seen in submandibular and parotid salivary glands, showing normal appearance.
- Normal appearance of the neck muscles & vessels.
- No hypermetabolic cervical lymphadenopathy detected.
- No FDG avid neck soft tissue mass lesions.

Chest:

- Left lower lobe anterior segment Non FDG avid atelectatic bands.
- No hypermetabolic mediastinal or axillary lymphadenopathy.
- No FDG avid lung nodules or masses detected.
- No metabolically active pericardial or pleural lesions detected.
- Physiologic FDG uptake is noted in the heart, showing normal shape.

Abdomen and Pelvis:

- Patent homogenously enhancing portal vein and its main branches as well as its main tributaries.

Name : Alia Abdul Salam Sayyid Nasr

Date : 29-12-2025 10:01

Branch : Nasrcity

Medical No. : 248576

Ref.Physician Prof : Goustav Roussey Egypt

Ins.Company : التأمين الصحي

- No FDG avid hepatic lesions , No intra-hepatic or extra-hepatic biliary radicle dilatation.
- Physiologic diffuse FDG uptake is seen over average size spleen with homogenous parenchymal texture and no FDG avid focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Physiologic FDG excretion is seen in the kidneys and bladder.
- Normal appearance of the pancreas, aorta, I.V.C and both supra-renals with no FDG avid lesions detected.
- No hypermetabolic abdominal or pelvic lymphadenopathy.
- No FDG avid ascites or abdominal collections noted.
- No metabolically active pelvic lesions.

Musculoskeletal:

- *Right femoral internal fixations is noted.*
- No osteolytic or sclerotic FDG avid lesions could be noted.
- Spondylodegertative changes of the dorso lumber vertebrae.

Conclusion:

- ± Left breast FDG avid mass with related FDG avid parenchyma associated with thickened low FDG skin, single level I locoregional axillary lymph node for further Triphasic CT / Triphasic MRI study.
- ± No distant metastasis could be noted.

Thank you for referral,



جمهورية مصر العربية
بطاقة تحقيق الشخصية

عليه

عبد السلام سيد نصر

٦ عبد السلام المنسى - كفر طهرمس - فيصل

بولاق الدكرور - الجيزة

١٩٥٨/٠٦



٢٥٨.٦.٣٢١.٠٠٠.٦٥

LM6795055

Dms

موصى فسيجو

ECHO REPORT

NAME: أحمد بن علي

DATE: 27/9/2026

ID NUMBER: /

SEX: AGE:

EF	62%	55-75%	PWT	0.8	0.6-1.1cm
LVEDD	4.6	3.5-5.8cm	AO.R	2.1	3-3.7cm
LVEDD	2.3	2.4cm	LA	3.5	2.4cm
FS	28%	25-45%	RV		0.7-2.3cm(supine)
SWT	0.9	0.6-1.1cm	AO.CUSP		1.5-2.6cm

LV Dimensions Normal

LV Wall Thickness Normal

LV Diastolic Function GI

SWMA Normal

LA and Ao Root Dimensions Normal

Mitral Valve Mild MR

Aortic Valve Normal

Tricuspid Valve TR

Pulmonary Valve

Right Side and Systolic Function TAPSE: 2.1

Intracardiac Masses and Effusion

DR.

Alshin